Recuperative Care Admission Criteria and Eligibility Outline	
General Admission Criteria	
Meets one or more of the following: Homeless/at risk of homelessness and too ill or frail to recover from illness or injury Lives alone with no formal supports and too ill or frail to recover from illness or injury At risk of hospitalization or re-hospitalization after hospitalization	
ADLs	Able to complete all Activities of Daily Living (ADLs) independently including personal hygiene Patient must be able to complete personal hygiene independently. Benches are available for bathing so seated shower is acceptable. Patient must be able to gather all of their own bathing/showering supplies, transfer to shower area and perform shower/bath independently Able to toilet independently and/or change briefs/diapers independently
Mobility	If using DME (wheelchair, walker, etc) must be able to use independently PT/OT notes must be sent for review Patient must be able to ambulate at least 100 ft (rest breaks are acceptable) at modified independence if not wheelchair dependent Wheelchair dependent patients must be able to transfer independently
Mental Health	Patient must be mentally stable on medications No suicidal ideation No commanding auditory or visual hallucinations If a patient was admitted with acute alcohol intoxication, the patient must be held for at least 72 hours and not have signs of alcohol withdrawal (maintain a CIWA-AR score of <8, if being assessed with this tool) prior to admission to recuperative care. Cannot qualify, have history of or currently be under a conservatorship or FSP
Cognition	Must be capable of self-administering medications (must confirm that patient can self-administer insulin) and receiving medical education AOx4
Other Factors	Ineligible if client is verbally or physically aggressive towards staff or other patients Patients that are registered sex offenders will be evaluated on a case-by-case basis (however, as a general guide, any offense that involves violence will likely be denied Patients with IV access can be accepted if they have home health established to maintain the IV and administer the medication as needed. There is no limit on time for IV Cannot accept patients on isolation unless isolation status is cleared prior to admission to recuperative care
End of life care	Hospice patients are eligible for recuperative care admission. Hospice services must be arranged prior to admission to recuperative care and patient, family and other involved parties must be made aware that recuperative care staff cannot administer medications, these medications will be administered by hospice staff
Infection Control	MRSA swab, documented negative prior to recuperative care admission if coming from inpatient hospitalization Covid swab (rapid swab acceptable) documented negative within 24 hours of recuperative care admission if coming from inpatient admission

Specific Disease Processes	
Diabetes	If patient is being referred from inpatient hospitalization, 5 consecutive glucose readings should be sent. Readings must be show improvement and stabilization. If admitted from the emergency department, it must be established and documented that the patient is not in DKA. We will not accept a diabetic patient that was started on an insulin regimen less than 24 hours prior to admission. Poorly controlled diabetics on an oral medication or established insulin regimen may be accepted from the ED if they have follow up with a primary care provider scheduled within the next 5-7 days, have not had an episode of hypoglycemia within the last week and have a written treatment plan for glucose control (i.e. sliding scale parameters, medication hold guidelines, etc). Patient should understand how to use glucometer Patient should understand insulin administration, if applicable **Associated Questions** Does the patient use insulin? Is the insulin administered through a pump? Does the patient take oral medication? What was the patient's last hemoglobin A1C? This is not a limiting factor for admission but does provide important information about the patient's baseline glucose level over
Infection	The last 5 sets of vital signs should be sent. Patient is ineligible for recuperative care if VS show signs of sepsis (continued fever, elevated heart rate, low blood pressure) The patient should be able to administer oral antibiotics independently If IV antibiotics are required, home health care should be ordered before admission to recuperative care to maintain IV and administer antibiotics **Associated Questions** Location of infection? Were cultures performed? Results should be sent Is the patient on isolation?
Respiratory Illness	Chest imaging should be sent (Chest XR, CT, MRI) when available The last 5 sets of vital signs should be sent, if available; O2 saturation may be chronically low if patient has a history of COPD, readings should be stable and consistent with patient's normal O2 saturation If O2 is required, the hospital must order it and have the tank ready to be sent to the site the day before the client arrives at the recuperative care site Respiratory therapy notes should be sent **Associated Questions** Is the patient receiving any breathing treatments? If so, what are they? Is the patient on antibiotics for this condition?
Cardiac	The last 5 sets of vital signs should be sent. Patient's BP should be stable on oral medication for at least 24 hours If patient had a troponin in the hospital, at least 3 consecutive results should be sent with at least 2 negative results EKG results should be sent If patient has a history of atrial fibrillation or has been diagnosed with atrial fibrillation, patient should be able to take anticoagulation therapy independently and understand any associated lab work (i.e. INR) necessary to track this medication Ejection Fraction of 25% or higher may be accepted as long as this has been established to be the patient's baseline. If the patient is on chemotherapy or is being immediately discharged from hospitalization for heart attack with a lowered ejection fraction, this patient is ineligible for recuperative care **Associated Questions** Has the patient required IV anti-hypertensive medications within the last 24 hours? Patient will not be accepted for admission to recuperative care if IV blood pressure medications have been required within 24 hours

Integumentary	**Eligible**
	Uncomplicated infections of the skin, cellulitis or abscess, requiring oral antibiotic treatments
	Wound care to be coordinated with home health agency prior to arrival at recuperative care and should coincide with the frequency of dressing changes or wound monitoring. If a patient should have a dressing change or wound check within 24 hours of discharge from the hospital, wound care should be arranged to arrive to recuperative care within 24 hours. Additionally, all wounds in hard-to-reach areas (coccyx, , back of thighs, back, perineum) will require at least one visit from wound care in the recuperative care setting regardless of patient's ability to change the dressing in a hospital setting.
	IV antibiotics are allowed as long as home health has been coordinated to maintain the IV and either patient is able to administer their own IV antibiotics or home health will be administering the antibiotic
	Uncomplicated wounds (e.g. laceration without infection)
	Post-surgical incision. If staples or external sutures are placed, appropriate follow up should be scheduled for removal prior to admission to recuperative care
	Stage 1, 2, 3 pressure ulcers. Wound care to be coordinated and scheduled prior to admission to recuperative care
	Third degree burns that affect <15% of the body. Wound care to be coordinated with home health agency prior to admission to recuperative care
	Ineligible
	Complicated wounds requiring skin grafting
	Complicated large abscess
	Burns associated with inhalation injury
Ophthalmology	**Eligible**
	Uncomplicated infections of the eye
	Post-op glaucoma surgery
	Cataract surgery requiring extraction with lens implant (uncomplicated situation)
	Diabetic retinopathy
	Diabetic macular edema
	Post operative admission for vitreous hemorrhage, if patient meets all other criteria
	Post operative admission for retinal detachment, if patient meets all other criteria
	Eligible
	Pneumonia
	Bronchitis
	COPD not requiring continuous oxygen therapy
	Asthma
	Patients with O2 baseline of 94% or greater with physician explanation

Respiratory	Patients with mature tracheostomy who are able to independently care for their tracheostomy site and are also able to eat and use a speaking valve
	CPAP
	Ineligible
	Pneumothorax
	Hemothorax
	Trach patients with ventilator
	Pulmonary Embolism
	Respiratory conditions requiring BiPAP
Musculoskeletal	**Eligible**
	Closed fractures
	PT to be coordinated and scheduled prior to arrival at recuperative care
	Rheumatoid arthritis
	Non-elective hip/knee replacements following SNF placement, if patient meets the other recuperative care guidelines
	Ineligible
	Open fractures
	Osteomyelitis
	External fixation devices
	Immediately following hospitalization for amputation with wounds
	Facial trauma with mandibular or maxillary fracture requiring jaw wiring
GI	**Eligible**
	Post abdominal surgical care
	Uncomplicated inguinal or umbilical hernia
	Diverticulitis
	Uncomplicated inflammatory bowel disease
	Ascites, if chronic
	Gastroesophageal reflux disorder
	Post hospitalization for uncomplicated bowel obstruction that did not require surgical intervention
	Ineligible
	Decompensated cirrhosis (defined by the development of clinically evident complications of portal hypertension, ascites, variceal hemorrhage, hepatic encephalopathy or liver insufficiency, jaundice)
	Immediately following hospitalization for bleeding ulcer disease or esophageal varices
	C.Diff
	Eligible
	Diabetes that shows improvement and stabilization in glucose levels as seen on at least 5 consecutive glucose readings with medical treatment (Please see diabetes criteria in "disease processes" for full guidelines)

Endocrine	Hypothyroidism or hyperthyroidism on medical treatment
	s/p thyroidectomy
	Ineligible
	Immediately s/p parathyroidectomy
	Alcoholic ketoacidosis
	DKA
GU	**Eligible**
	Urinary tract infection
	Post-acute glomerulonephritis, if caused by an infection
	Post-op laparoscopic gynecologic surgery for treatment of non-cancerous conditions
	Urolithiasis
	Patients requiring catheterization if confirmation is provided that patient is able to self-catheterize
	Indwelling catheter if patient is able to empty on their own and follow up has been scheduled for removal
	Patients requiring dialysis if dialysis schedule and frequency has been established
	Ineligible
	Patients requiring intermittent catheterization that has to be performed by medical staff
Circulatory System	**Eligible**
	Post-op angiogram
	Cardiac catheterization/stent placement
	Pacemaker/ICD insertion
	Stable hypertension (patient with baseline elevated BP; SBP<170, DBP<100, asymptomatic and not currently taking medication but with established follow up with a primary care provider)
	Post coronary artery bypass graft or heart valve surgery with controlled, stable blood pressure, normal hemoglobin/hematocrit, normal EF and without arrhythmias. Home health must be ordered and scheduled prior to admission to recuperative care. Cardiac surgery follow up must be scheduled prior to admission to recuperative care.
	Hypertension that responds well to oral anti-hypertensive regimen
	History of heart attack. If being admitted to recuperative care immediately following admission for heart attack, will only accept patients who have an normal ejection fraction
	Ineligible
	Unstable cardiac arrhythmias
	Hypertension that requires IV medications to improve, is symptomatic or is not responding to oral medication
	Eligible
	Alcohol withdrawal seizure, following detox/stabilization
	Post carpal tunnel or cubital release
	Mild traumatic brain injury with intact ADLs

Neurology	Cervical or lumbar radiculopathy following epidural injection or decompression surgery
	Ineligible
	Stroke syndrome with neurological residual/deficit (e.g. ambulatory difficulty, seizure)
	New onset of seizure with neurological deficit/impairment
	Moderate or severe traumatic brain injury
	Cervical or lumbar fusion
	Dementia
	Immediately s/p neurosurgery. Neurosurgical patients who meet other eligibility criteria should be escalated to further clinical review